REGIONAL SURGICAL CENTER AT BOSTON

45 Surgical Drive, Boston, MA 02115 | Tel: (617) 555-0220

OPERATIVE REPORT

Patient:	Brianna Nicole Foster	**MRN:**	RSCB-228831
DOB:	09/03/2001 (Age 22)	**SSN:**	483-61-9920
Date of Surgery:	07/19/2023	**Pre-op Dx:**	Acute appendicitis
Address:	89 Commonwealth Ave, Apt 12, Boston, MA 02116	**Phone:**	(617) 555-3381
Surgeon:	Dr. Thomas Osei, MD, FACS	**Anesthesiologist:**	Dr. Yuki Tanaka, MD
Scrub Tech:	Michael Brennan, CST	**Circulator:**	Donna Walsh, RN
Insurance:	Aetna Student Health #AET-STU-7741	**Hospital #:**	RSCB-2023-07192

PRE-OPERATIVE DIAGNOSIS

Acute appendicitis

POST-OPERATIVE DIAGNOSIS

Acute suppurative appendicitis without perforation

PROCEDURE PERFORMED

Laparoscopic appendectomy

ANESTHESIA

General endotracheal anesthesia. Patient intubated without difficulty. Intraoperative course uneventful from anesthesia standpoint.

DESCRIPTION OF PROCEDURE

The patient was brought to Operating Room 4 at 14:22. After induction of general endotracheal anesthesia by Dr. Tanaka, the patient was positioned supine. Foley catheter placed. The abdomen was prepped with chlorhexidine and draped in standard sterile fashion.

A 10 mm umbilical incision was made and the Hasson technique was used to establish pneumoperitoneum to 15 mmHg CO2. A 10 mm trocar was placed at the umbilicus and two additional 5 mm trocars were placed in the left lower quadrant and suprapubic regions under direct visualization.

Laparoscopic exploration revealed a moderately inflamed, non-perforated appendix with periappendiceal fat stranding. No free fluid or abscess noted. The mesoappendix was taken down with LigaSure device. The appendiceal base was doubly ligated with endoloops and divided. The appendix was placed in a retrieval bag and removed via the umbilical port.

Thorough irrigation of the right lower quadrant with 500 mL warm saline. Hemostasis confirmed. Trocars removed under direct vision. Pneumoperitoneum released. Fascial defect at umbilicus closed with 0-Vicryl. Skin closed with 4-0 Monocryl subcuticular suture. Steri-strips applied. Sterile dressings placed.

Patient tolerated procedure well. EBL approximately 15 mL. Sponge, needle, and instrument counts correct x2. Patient taken to PACU in stable condition.

SPECIMENS

Appendix — sent to pathology. Specimen labeled: 'Appendix, Brianna Foster, 07/19/2023.' Gross: 6 cm vermiform appendix, markedly congested and hyperemic serosa. Cut section shows mucosal erythema without fecalith.

COMPLICATIONS

None.

DISPOSITION

Patient transferred to PACU at 15:48 in stable condition. Anticipated discharge within 23 hours pending tolerating PO and adequate pain control.

Surgeon: Dr. Thomas Osei, MD, FACS | NPI: 2233445566 | Dictated: 07/19/2023 16:05 | Transcribed: 07/19/2023 17:30